BHAT FOUNDATION WELFARE SOCIETY

PROJECT COMPLETION & IMPACT REPORT

FREE MEDICAL CAMP

For the People of Bemina



Date	03 May 2026
Venue	Rakh-I-Arath Colony, Bemina
Organised by	Bhat Foundation Welfare Society
In collaboration with	Ibn Sina Hospital, Ompora Budgam

1. Executive Summary

Bhat Foundation Welfare Society, in collaboration with Ibn Sina Hospital, conducted a Free Medical Camp for residents of Rakh-I-Arath Colony, Bemina, on 03 May 2026. The documented purpose of the intervention was to provide accessible healthcare services and social-welfare support to the local community.

The supplied event documentation records 500 patients receiving free consultation, including free medicine. The camp brought healthcare personnel and community members together at a local venue, with photographs showing registration/interaction areas, clinical consultation, health discussions and community participation.

The intervention addressed a practical access gap by bringing consultation and basic medical support closer to community members. Reported outcomes include increased access to healthcare, support for early identification of health concerns, improved health awareness, reduced immediate consultation-related financial burden, and referral guidance for people requiring further treatment.

The report distinguishes documented facts from photo-based observations. No unsupported beneficiary demographics, budget figures, volunteer counts, clinical diagnoses, or individual testimonials have been added.

Impact at a Glance

Indicator	Documented value
500	Patients recorded as receiving free consultation including free medicine
1	Community medical camp conducted
1	Locality documented: Rakh-I-Arath Colony, Bemina
1	Healthcare collaboration documented: Ibn Sina Hospital
03 May 2026	Documented event date
6	Core reported service/impact areas: consultation, examination, awareness, early identification, financial relief and referral guidance

2. Project Background

Community-based medical outreach can reduce practical barriers to initial consultation, particularly where residents may face cost, distance or access constraints. The supplied event report identifies free healthcare and social welfare as the intervention objective.

The camp was implemented at Rakh-I-Arath Colony, Bemina, using a collaboration model between the Bhat Foundation Welfare Society and Ibn Sina Hospital. The available documentation establishes the location, date, partner and patient count, but does not provide a formal baseline survey or community needs assessment. Accordingly, this report does not claim a quantified pre-intervention health gap.

3. Problem Statement

Problem → Need → Intervention

Problem: Community members may require accessible medical consultation and examination but can face immediate barriers to obtaining such services.

Need: A local, no-cost medical outreach mechanism was required to connect residents with healthcare personnel and initial medical guidance.

Intervention: Bhat Foundation Welfare Society, with Ibn Sina Hospital, organised a free medical camp at Rakh-I-Arath Colony, Bemina, providing consultation and reported free medicine to 500 patients.

4. Project Objectives

- Provide free medical consultation to community members.
- Facilitate access to basic medical examination and healthcare guidance.
- Support early identification of health concerns through consultation and examination.
- Increase awareness of preventive healthcare and the importance of appropriate follow-up.
- Reduce the immediate financial burden associated with consultation and reported medicines.
- Guide patients requiring further treatment toward appropriate referral or follow-up.

5. Target Beneficiaries

The event documentation identifies the general public/community as the target beneficiary group. A verified age, gender, disability, income or vulnerability breakdown was not supplied; the photographs visibly show women, men and children among participants, but these images are not sufficient to quantify demographic categories.

Category	Number	Basis	Direct/Indirect
General community/public	500 patients	Event report	Direct

6. Geographic Coverage

Geographic element	Documented information
District/area	Bemina, Srinagar area
Locality	Rakh-I-Arath Colony
Venue	Rakh-I-Arath Colony, Bemina
Reach	500 patients documented in the event report

7. Implementation Methodology

Stage 1 – Planning: The NGO coordinated the community medical outreach and prepared the local venue for the camp.

Stage 2 – Community mobilisation: Community members attended the camp; the supplied photographs show a substantial flow of participants and consultation activity.

Stage 3 – Stakeholder coordination: The intervention was conducted in collaboration with Ibn Sina Hospital.

Stage 4 – Team deployment: Healthcare personnel and supporting staff were present for consultation and patient interaction, as visible in the photographs.

Stage 5 – Main intervention: Patients received free consultation; the event report also records free medicine.

Stage 6 – Follow-up/referral: The event documentation reports guidance/referrals for patients requiring further treatment or surgical intervention.

Stage 7 – Documentation: Photographs, event information and the camp poster provide supporting evidence of the activity.

8. Activities Conducted

- Community-level free medical consultation.
- Patient interaction and preliminary medical assessment/examination.
- Distribution/provision of free medicine as documented in the event report.
- Health-related guidance and awareness during consultations.
- Identification of patients who may require further care.
- Referral guidance for patients requiring additional treatment or surgical intervention.
- Photographic documentation of the camp and community participation.

9. Outputs Achieved

Output indicator	Documented result	Evidence basis
Medical camp	1 camp	Event report and photographs
Patients served	500	Event report
Free consultation	Provided	Event report and photographs
Free medicine	Included in the reported service	Event report
Healthcare partnership	Ibn Sina Hospital	Event report and camp poster

10. Outcomes and Early Impact

The following are reported outcomes from the supplied event documentation. They should be understood as immediate/short-term results rather than independently measured long-term health outcomes.

- Improved access to free medical consultation for 500 documented patients.
- Support for early identification and diagnosis through examinations/consultations, as reported in the event documentation.
- Increased health awareness among participating community members, as reported.
- Reduced immediate financial burden through free consultation and reported free medicine.
- Patients requiring additional treatment or surgical intervention were reportedly guided toward appropriate referrals.
- The activity strengthened community-level healthcare outreach through NGO–hospital collaboration.

11. Evidence & Verification Register

Evidence item	What it supports	Status
Event report	Date, venue, partner, objective and 500-patient figure	Available
Camp poster	Event branding, date, venue and collaboration	Available
Photographs	Visible consultation, patient interaction, medical staff and community participation	Available

Attendance/demographic data	Age/gender/vulnerability breakdown	Not supplied
Budget/expenditure records	Financial utilisation/cost per beneficiary	Not supplied
Testimonials	Verified individual beneficiary quotations	Not supplied

12. Beneficiary Stories & Testimonials

No verified individual beneficiary testimonial was supplied with the event documentation. No quotation or personal story has been created for this report.

[INFORMATION REQUIRED: Add a consented beneficiary story/testimonial if available.]

13. Partnerships & Stakeholder Engagement

The event was organised by Bhat Foundation Welfare Society in collaboration with Ibn Sina Hospital. The partnership combined community-level mobilisation and event coordination with healthcare-sector participation.

The supplied poster and photographs provide visual evidence of the collaboration and medical camp activity. No additional institutional or CSR funder has been identified in the supplied material.

14. Resource Utilisation

Documented/visible resources included healthcare personnel, consultation arrangements, a community venue, patient interaction areas and event documentation materials.

Financial expenditure: [INFORMATION REQUIRED]

Volunteer/team count: [INFORMATION REQUIRED]

Medicine quantity/value: [INFORMATION REQUIRED]

Medical equipment/test quantities: [INFORMATION REQUIRED]

Transport/logistics cost: [INFORMATION REQUIRED]

15. Budget Utilisation & Cost-Effectiveness

No approved budget, expenditure statement or itemised financial record was supplied. Therefore, this report does not fabricate a project cost or cost-per-beneficiary figure.

Once expenditure records are available, the following indicator can be calculated: Total verified project expenditure ÷ 500 documented patients = verified cost per patient served.

16. Monitoring & Evaluation Framework

Indicator	Baseline	Result	Verification
Patients served	Not supplied	500	Event report
Camp conducted	0 for this intervention	1	Event report/photos

Consultation provided	Not supplied	Free consultation provided	Event report/photos
Referral tracking	Not supplied	Referral guidance reported	Event report

17. SDG Alignment

- SDG 3 – Good Health and Well-being: the intervention directly concerned access to healthcare consultation and health-related guidance.
- SDG 10 – Reduced Inequalities: free community-level healthcare access can help reduce immediate financial barriers to consultation.
- SDG 17 – Partnerships for the Goals: the activity was implemented through collaboration between a civil-society organisation and a healthcare institution.

18. Challenges, Lessons & Follow-Up

Specific operational challenges were not documented in the supplied materials. The main documentation lesson is the importance of maintaining standardised registration, beneficiary demographics, referral tracking, consented feedback/testimonials and complete expenditure records for future camps.

Follow-up outcome data: [INFORMATION REQUIRED].

19. Theory of Change

INPUTS → NGO coordination + healthcare partnership + community venue + medical personnel

ACTIVITIES → Community mobilisation + medical consultation/examination + medicine provision + health guidance + referral guidance

OUTPUTS → 1 documented camp + 500 patients served

SHORT-TERM OUTCOMES → Improved access to consultation, health guidance and reported financial relief

LONG-TERM IMPACT → Intended contribution to improved community access to healthcare; long-term health impact was not independently measured in the supplied evidence.

20. Donor / CSR Value Proposition

The documented intervention demonstrates a practical community-health outreach model in which local mobilisation is combined with healthcare-sector collaboration. The principal verified service-delivery indicator is 500 patients receiving free consultation including free medicine, according to the supplied event report.

For future donor or CSR-supported camps, impact accountability can be strengthened through standardised beneficiary registration, demographic reporting, referral tracking, patient feedback, treatment follow-up and transparent project expenditure reporting.

21. Photographic Documentation

The photographs below are included as supporting evidence. Captions describe only objectively visible activity and do not infer clinical diagnoses or demographic totals from images.



Photo 1 — Community participants gathered during the medical camp, with consultation activity visible in the venue.



Photo 2 — Healthcare personnel and community members engaged in consultation and patient interaction.



Photo 3 — A consultation/registration interaction taking place with community participants.



Photo 4 — Medical staff attending to community members during the outreach activity.



Photo 5 — Patients and healthcare personnel present during the camp; the event poster is visible in the background.



Photo 6 — Community members seated and waiting/participating in the medical outreach activity.



Photo 7 — Consultation and patient-registration activity taking place around the service desk.

22. Conclusion

The Free Medical Camp at Rakh-I-Arath Colony, Bemina, on 03 May 2026 represents a documented community-health outreach intervention by Bhat Foundation Welfare Society in collaboration with Ibn Sina Hospital. The supplied event record reports 500 patients receiving free consultation including free medicine.

The available evidence demonstrates service delivery, healthcare personnel participation and community engagement. Reported outcomes include improved access to consultation, health awareness, reduced immediate financial burden and referral guidance. Further beneficiary-level and financial records would allow stronger measurement of longer-term impact and cost-effectiveness.

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